

Undergraduate Medicine

Obstetrics & Gynecology Study Pack

CharaNas Medicine Bot — Specialty Notes

Expanded notes with original schematic figures for revision. These are educational drawings inspired by standard undergraduate teaching themes, not copied textbook plates. Always follow your faculty curriculum and latest guidelines.

Section	Focus
1	Pregnancy basics
2	Obstetric emergencies
3	Early pregnancy problems
4	Gynecology essentials
5	Checklist & books
6	Quick pearls from the bot notes
7	Recommended book sources

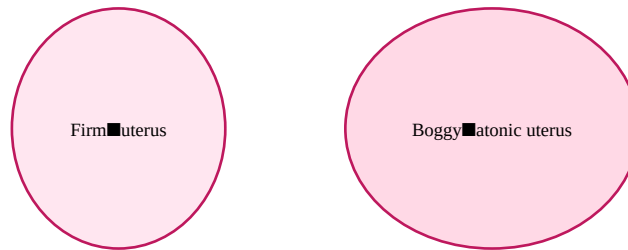
1. Pregnancy basics

Confirm pregnancy with β -hCG; date by LMP and early ultrasound. Routine antenatal care screens for anemia, blood group/Rh, infections, and fetal anomalies per schedule.

Physiologic changes: increased blood volume, hypercoagulability, reduced residual volume, delayed gastric emptying — important for drugs and anesthesia risk.

Fetal heart often audible by Doppler around 10–12 weeks.

Figure: Uterine atony concept after delivery



Atony → failure to contract → postpartum hemorrhage (most common PPH cause)

Schematic figure for learning — verify details in your recommended textbooks.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about obstetrics & gynecology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

2. Obstetric emergencies

Postpartum hemorrhage: 4 Ts — Tone (atony most common), Trauma, Tissue (retained placenta), Thrombin (coagulopathy). Uterine massage + oxytocin are first steps for atony.

Eclampsia/pre-eclampsia: HTN after 20 weeks with proteinuria or organ dysfunction; severe features need urgent care; magnesium for seizure prevention/treatment per protocol.

Shoulder dystocia, cord prolapse, and maternal collapse have drill-based responses — know your local obstetric emergency algorithms.

Revision prompts

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3. Early pregnancy problems

Ectopic pregnancy: positive hCG, pain/bleeding, empty uterus — can rupture. Prior PID/tubal surgery increases risk. Unstable patients need urgent surgery pathways.

Miscarriage spectrum: threatened, inevitable, incomplete, complete, septic — assess bleeding, pain, hemodynamics, and products of conception.

Molar pregnancy may present with exaggerated symptoms and characteristic ultrasound appearances.

Revision prompts

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- Name 2 investigations and 2 initial management steps you would consider first.

4. Gynecology essentials

Abnormal uterine bleeding: structure vs non-structure framework (PALM-COEIN). Exclude pregnancy first in reproductive-age patients.

PID: pelvic pain, cervical motion tenderness, fever — treat early to reduce infertility/ectopic risk; consider TOA if severe.

Contraception counseling should cover efficacy, contraindications (e.g., estrogen and migraine with aura/thrombosis risk), and emergency contraception options.

Revision prompts

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- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

5. Checklist & books

Checklist: pregnancy test, vitals, abdominal/pelvic exam as appropriate, fetal heart when viable pregnancy, ultrasound, bloods (FBC, group & save in bleeding), escalate PPH/eclampsia early.

Books: Williams Obstetrics; Beckmann & Ling; Dutta Obstetrics.

Revision prompts

- Explain this section aloud in 60 seconds as if teaching a junior student about obstetrics & gynecology.
- List 3 red flags that would make you escalate care urgently.
- Name 2 investigations and 2 initial management steps you would consider first.

6. Quick pearls

High-yield reminders packaged with this specialty in the CharaNas bot:

1. PPH 4 Ts: Tone (atony most common), Trauma, Tissue, Thrombin.
2. Ectopic: positive hCG + empty uterus + pain/spotting — emergency awareness.
3. Pre-eclampsia after 20 weeks: HTN + proteinuria/organ dysfunction.
4. Antenatal care milestones: dating, anomaly scan, Rh, vaccines as indicated.
5. Doppler FHR typically ~10–12 weeks.
 - Link symptoms → anatomy/physiology → investigation → first management step.
 - Write one OSCE-style explanation for a common presentation in this specialty.
 - After reading, do the bot Short MCQ and Case-based Question for active recall.

7. Recommended book sources

Standard undergraduate references commonly used internationally. Use the edition your college recommends. Figures in commercial textbooks are copyrighted; use library/licensed access for full plates and photographs.

#	Reference
1	Williams Obstetrics
2	Beckmann and Ling's Obstetrics and Gynecology
3	DC Dutta's Textbook of Obstetrics

How to study with this PDF: read one section → sketch the figure from memory → answer bot MCQs/cases → review mistakes → revisit the matching section.

Disclaimer: educational aid only; not a substitute for clinical guidelines, faculty teaching, or licensed textbooks.